7.12 Bladder Cancer and Urinary Diversion*

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • "I saw blood in my urine but felt no pain." • "Is it cancer?" **Objective** • Painless hematuria • Smoked for 30 years • Waiting for cystoscopy results	**ACTUAL** Excessive anxiety related to a possible cancer diagnosis and treatment as evidenced by worry and repeated questioning.	• Voices his concerns and reports less anxiety before treatment.	• Give him time to talk about his fears. • Provide clear information together with the medical team. • Support him to stop smoking. • Involve the family as he wishes.	• Considered met when he feels informed and calmer.
Subjective • "How do I look after the urostomy?" **Objective** • New ileal conduit after cystectomy • Urine flows from the stoma continuously	**ACTUAL** Inadequate health knowledge related to urostomy care as evidenced by questioning and inability to describe care.	• Before discharge, changes the appliance and connects night drainage on his own.	• Teach him to measure the stoma and change the appliance. • Show him how to connect a night drainage bag. • Teach a fluid intake of 2–2.5 L a day. • Explain that mucus in the urine is normal with an ileal conduit. • Teach him to report a dark stoma, fever or flank pain.	• Goal reached when he demonstrates correctly.
Subjective *No specific complaint* **Objective** • Urinary diversion • Ureteric stents	**RISK** Risk for infection as evidenced by risk factors: urinary diversion and ureteric stents.	• No UTI.	• Handle stents and drainage aseptically. • Watch temperature and the appearance of the urine. • Encourage fluids.	• Met if no infection develops.

* **References for this care plan:** [28], [32], [35], [37]. Full entries are listed in the References section of the CPMN book.